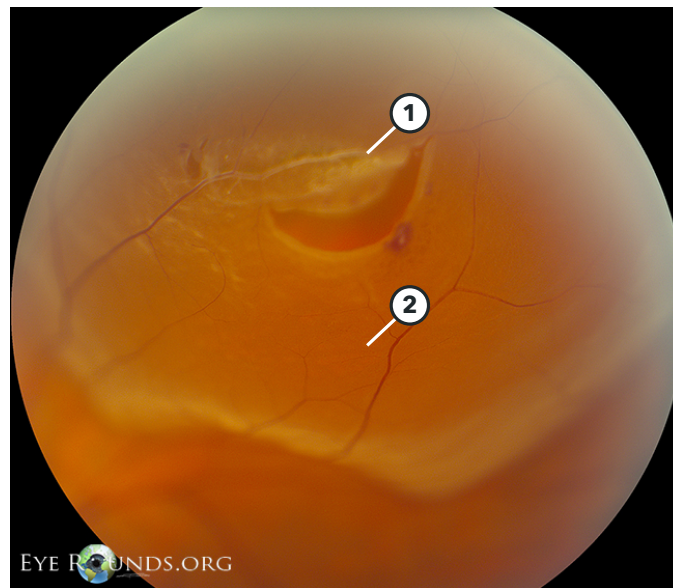


Retinal tears, lattice and detachment mechanisms

Clinical M3-M5 | Focused Type B notes

A symptomatic tear is a prevention opportunity

A horseshoe tear with surrounding elevation



1: full-thickness tear. 2: surrounding elevated retina. This example has detachment; an earlier tear with attached retina may be treated before fluid extends. Supplied M3 Anki, media 161.

Different peripheral findings

Finding	Usual treatment direction
Acute symptomatic horseshoe tear +traction	Prompt retinopexy when feasible.
Established clinical detachment	Vitreoretinal assessment for repair.
Uncomplicated asymptomatic lattice, no tear/fluid	Observation, warning-symptom education and risk-based review.

Laser needs contact

Retinopexy creates adhesion around a tear in attached tissue. It is not a universal substitute for reattaching a detached retina, and every asymptomatic lattice patch does not require prophylactic laser.

References AAO: PVD, Retinal Breaks and Lattice PPP, 2025 | AAO: Retinal detachment

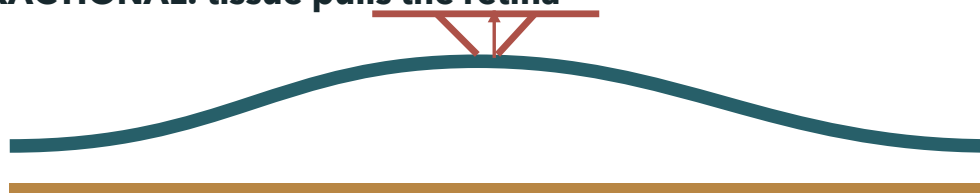
Break, pull or leakage?

Three routes into the subretinal space

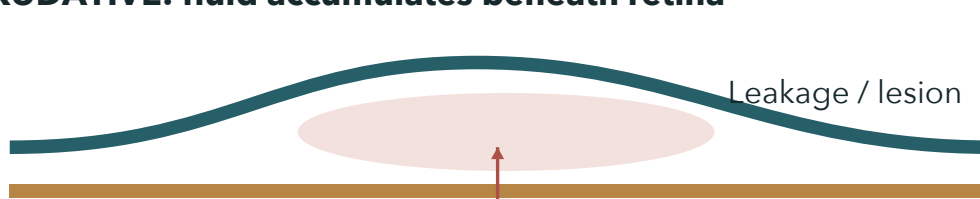
RHEGMATOGENOUS: fluid through a break



TRACTIONAL: tissue pulls the retina



EXUDATIVE: fluid accumulates beneath retina



Original cross-sections: teal is retina; gold is RPE. A break is required for rhegmatogenous detachment; tractional and exudative forms can occur without one.

Choose a cause-directed plan

Rhegmatogenous disease requires sealing the break with appropriate retinal reattachment. Tractional disease requires assessment of contracting tissue, especially near the macula. Exudative disease requires investigation and treatment of the inflammatory, choroidal or tumour-related cause.

Mixed mechanisms exist

Fibrovascular traction can create a break and produce combined tractional-rhegmatogenous detachment. Use examination and imaging rather than forcing a complex eye into a single mechanism from the history alone.